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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°148 /MVE-BDBV/09/10/2026

Report date: October 9, 2026 Publication date: October 10, 2026

HIGHLIGHTS (last 24 hours)

- Sixty-nine (69) new confirmed cases of Ebola Virus Disease (EVD), including 30 community deaths, have been notified. These cases came from the provinces of Ituri (47 cases), North Kivi (17 cases), Tshopo (2) and Haut Uele (1) and at Bas Uele (2). In addition, 10 deaths were recorded in the CTEs. A total of 431 new samples were analyzed; 69 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 16.0%.

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
8914	4 315	48,4 %	946	2 338	83,1%

Affected provinces and affected health zones

7 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

65 out of 167 health zones affected (38,9%)

Summary of key points from the last 24 hours

As of October 9, 2026, 69 new confirmed cases of Ebola Virus Disease (EVD) including 30 community deaths were reported in 5 provinces: Ituri (47 cases), North Kivu (17 cases), Tshopo (2 cases), Bas-Uélé (2 cases) and Haut-Uélé (1 case).

In addition, 10 deaths were recorded in the CTEs (4 in Ituri and 6 in North Kivu), bringing the total number of confirmed deaths for the period in question to 40.

On the other hand, 18 patients, all in Ituri, were declared cured after obtaining two negative results in the control test for Ebola virus disease.

For the day of October 9, 2026, the DRC has accumulated **8914** confirmed cases and **4315** deaths, representing a case fatality rate of **48.4%.** Ituri remains the epicenter of the epidemic with **74.1** % of cumulative confirmed cases and **68.1%** of new confirmed cases reported in the last 24 hours.

65 out of 167 health zones (38.9%), spread across seven provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 9, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	47	6609	3 065	46,4%	28/36 (77,8 %)
North Kivu	17	1858	1 065	57,3%	17/34 (50,0 %)
Haut-Uélé	1	372	157	42,2%	7/13 (53,8 %)
Boss	2	57	22	38,6%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	2	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	**69**	**8 914**	**4 315**	**48,4%**	**65/167 (38,9%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 9, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6609**	**3065**	**46,4%**	**47**	**25**	**4**	**29**
Give	12	1	8,3%				0
Understand	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	213	47	22,1%				0
have	2	2	100,0%				0
Bunia	1804	601	33,3%	6	1	1	2
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	9	5	55,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	176	107	60,8%				0
Liter	297	150	50,5%	2	1	0	1
Logo	14	7	50,0%				0
Fight	35	9	25,7%				0
distributed	4	1	25,0%				0
Washing	41	17	41,5%				0
Darkness	235	123	52,3%	18	14	1	15
Mangala	369	160	43,4%	3	3		3
Mongbwalu	710	339	47,7%	0	0	1	1
Nanny	320	181	56,6%	3	3		3

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Nizi	806	290	36,0%	4	1		1
Legumes	133	39	29,3%				0
Jungle	18	5	27,8%	4	0	0	0
Rwampara	1128	391	34,7%	7	2	1	3
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT!				0
North Kivu	**1858**	**1065**	**57,3%**	**17**	**5**	**6**	**11**
Me	437	293	67,0%	7	5	5	10
Farm	23	7	30,4%				0
Butembo	295	185	62,7%	2	0		0
Ten	1	0	0,0%				0
Kalunguta	67	24	35,8%	1	0		0
Cut	620	379	61,1%	1	0		0
Source	3	2	66,7%				0
Kyondo	37	20	54,1%				0
Lubero	23	11	47,8%				0
Be anxious.	56	21	37,5%	1	0		0
Alimbong	5	3	60,0%				0
Manguredjipa	13	5	38,5%				0
Masereka	36	11	30,6%				0
The museums	142	69	48,6%	1	0		0
Mutwanga	23	4	17,4%	3	0		0
Oicha	38	18	47,4%	1	0		0
Vuh	39	13	33,3%			1	1
Upper Uele	**372**	**157**	**42,2%**	**1**	**0**	**0**	**0**
Government Mangbetu	44	19	43,2%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	102	44	43,1%				0
Lining	98	47	48,0%	1	0	0	0
Beans	4	2	50,0%				0
Ordinary	120	43	35,8%				0
Boss	**57**	**22**	**38,6%**	2	0	0	0
Bafwasende	7	1	14,3%				0
Kabondo	6	1	16,7%	1	0	0	0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	30	12	40,0%	1	0	0	0
Mangobo	10	7	70,0%				0
Boss	2	0	0,0%				0
Wanie - Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	2	0	0	0

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Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%	2	0	0	0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Total	**8914**	**4315**	**48,4%**	**69**	**30**	**10**	**40**

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for October 9, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	104	6	110	15	5	0	0	20	12
Bas Uélé	139	2	141	16	2	123	0	18	0
Ituri	1 194	84	1278 111		84	1 083	0	195	79
North Kivu	1 411	56	1467 173		56	1 238	0	229	128
Sud Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Boss	153	0	153	11	0	142	0	11	0
Total General	**3 001**	**148**	**3 149 326**		**147**	**2 586**	**0**	**473**	**219**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

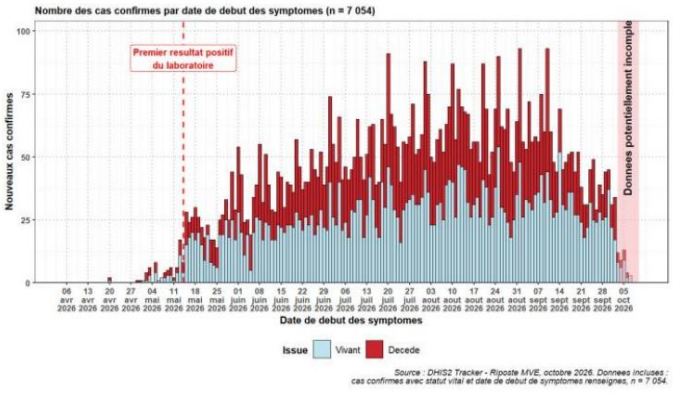
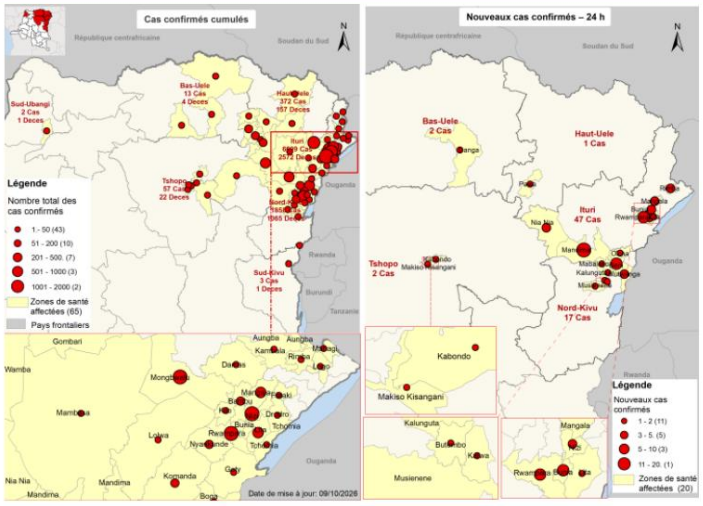


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



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1.COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

• As of October 9, 2026, 3,149 alerts were reported in five provinces. 3,059 were verified (97.1%), of which 473 (15.5%) were validated as suspected cases and 2,586 (84.3%) were invalidated. All 473 suspected cases were investigated (100.0%). Among them, 219 (46.3%) were transferred to COVID-19 treatment centers.

• Of the 31,476 contacts being monitored, 26,170 were seen in the last 24 hours, representing a follow-up rate of 83.1%, down from the previous day (83.5%) and below the required threshold of 95.0%. By province, this rate was 91.0% (600/659) in Tshopo, 90.5% (1,964/2,170) in Haut-Uélé, 84.8% (189/223) in Bas-Uélé, 84.1% (13,659/16,251) in Nord-Kivu and 80.2% (9,758/12,173) in Ituri; Sud-Ubangi did not submit its data (ND).

No province reaches the 95% threshold.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

• Four (4) alerts were notified to the PoE/PoC on October 9: three (3) in Ituri, one (1) in North Kivu, and one (1) in Haut-Uélé. In Ituri, the live alert validated at the Mabakese PoC (Mambasa Health Zone) was not referred to the Mambasa CTE due to a lack of available space, and two bodies intercepted at the Watsa Road PoC (Aru Health Zone) were swabed by the EDS team (results pending). In North Kivu, the live alert notified to the Kahongo PoC was invalidated after further investigation.

Table 4. Status of key indicators for PoE/PoC by province, as of October 9, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	95,1% (58/61)	95,2% (20/21) 100,0% (18/18)		96,0% (96/100)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	138 913	87 583	21 210	247 706
% of travelers screened	97,9%	97,8%	98,5%	97,9%
% of travelers who washed their hands	97,5%	97,8%	98,3%	97,7%
% of travelers made aware	98,7%	97,8%	100,0%	98,5%
Number of alerts notified	3	1	0	4
Number of validated live alerts (suspected cases)	1	0	0	1
Number of bodies recovered today	2	0	0	2
% of suspected cases transferred to CTE/CT	0,0%	THAT	THAT	0,0%
% of lifeless bodies swabed/secured	100,0%	THAT	THAT	100,0%
% of suspected cases investigated within 4 h	100,0%	THAT	THAT	100,0%

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2.2.2. Challenges

- In Tshopo, 1.5% of travelers (317/21,210) refused screening and 1.7% (369/21,210) refused handwashing, amid rumors that "the water contains the virus" or "the disease does not exist." In Ituri, the Points of Control (PoCs) in Avakubi and Zunguluka are not operational due to insecurity, and the Biakato Mayi PoC has not reported any data. In North Kivu, the Butuhe PoC remains non-operational since the security incident of September 26, 2026. In South Kivu, the reporting completeness rate is only 55.6% (5/9), and in Haut-Uélé, it is 70.0% (7/10). In Bas-Uélé, only 19 PoCs out of 41 notification sites are functional (46.3%), no PoE is reinforced (0/11), the completeness of reports is 72.2% (13/18) and the PNHF services are not operational.

2.3. Laboratory

2.3.1. Main actions

- Ituri: 47 new positive results (22 living and 25 deceased) out of 218 samples received and analyzed (positivity: 21.6%).
- North Kivu: 17 new positive results (12 living and 5 dead) out of 156 samples received and analyzed (positivity: 10.9%).
- Haut-Uélé: 1 new positive result (1 live sample) out of 13 samples received and analyzed (positivity: 7,7%).
- Tshopo: 2 new positive results (2 blood tests) out of 11 samples received and analyzed (positivity) : 18,2%).
- Bas Uélé: 2 new positive results (2 blood tests) out of 33 samples received and analyzed
- (Positivity: 6.1%).

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- Ituri: 22 out of 34 rings were opened (64.7%), 46 out of 55 households were decontaminated (83.6%), and 4 out of 4 health facilities were decontaminated (100%); no households, social economy enterprises, or schools received kits. Of 91 SDS alerts (including 10 postponements from the previous day), 57 SDS were carried out (62.6%) and 63 bodies were swabed, with 26 failures (28.6%) due to resistance in Komanda, Nyakunde, Mongbwalu, Nia-Nia, Fataki, and Mandima.
- North Kivu: 26 out of 46 rings were opened (56.5%), with a significant gap in Katwa and Beni; 29 out of 33 health centers (87.9%) were decontaminated, and 18 out of 54 households (33.3%) were decontaminated. None of the decontaminated households received kits. 16 infection prevention and control (IPC) assessments (scorecard) were conducted in 16 health centers with an average score of 33.2%, and 3 infected primary wards (PLU) out of 30 confirmed cases (10.0%) were reported (2 in Katwa and 1 in Butembo). Out of 87 expected deaths (36 alerts and 51 postponements), 50 death assessments (DAS) were carried out (64.9%), and 36 bodies were successfully swabbed, with 37 postponements to the following day. Handwashing facilities have been installed at EP BEU and 3 classrooms have been decontaminated in 3 schools in the Beni health zone.
- Haut-Uélé: 3 out of 3 planned ESS (Emergency Situations) were decontaminated in the Isiro Health Zone (100%), 15 ESS were supervised and 9 were assessed using the scorecard method (7 in Isiro and 2 in Pawa), none reaching 80%, 66 out of 77 service providers (85.7%) were briefed and 2 Category 3 IPC (Infection Prevention and Control) kits were provided to the Maman Kahenga and Tshakatshaka Health Centers (Wamba Health Zone); no ring was opened around the 3 cases from Bolebole. Of 5 EDS (Emergency Discharge Summary) alerts, 3 EDS were carried out (60.0%) and 5 bodies were swabed, with 2 EDS refusals in the N'Sele Health Area (Isiro Health Zone).

2.4.2. Challenges

- In Ituri, the opening of the rings remains insufficient (64.7%), there are insufficient teams in Komanda, Mangala and Mandima, mobility is lacking in Mambasa, Nizi and Mandima, ECUMER coverage and EDS inputs are insufficient and 26 resistances to EDS have been recorded.
  In North Kivu, the gap in the opening of the rings in Katwa and Beni, the low rate of household decontamination (18/54; 33.3%), the lack of distribution of kits to decontaminated households, and the high-risk exposure of 4 out of 6 staff members assessed in Butembo persist. In Bas-Uélé, 16 out of 20 health establishments lack functional triage, waste, garbage, and PPE areas are lacking, and the Dakwa health zone urgently needs infection prevention and control (IPC) supplies. In Haut-Uélé, only 36

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ZS out of 7 have transmitted their PCI data, PCI kits are lacking for ESS and households, only 1 out of 15 ESS has a functional triage circuit and the distance (180 km) between Wamba and Bolebole hinders decontamination around cases.

2.4.3. Holistic care

2.4.4. Main actions

- In Ituri, 81 new admissions were recorded compared to 86 discharges, including 18 recoveries. 316 patients remained in isolation at the end of the day, representing a bed occupancy rate of 30.6 %: (316/1015), including 30 critically ill patients; 325 out of 330 patients (98.5%) received a hot meal, and all children under LNPE/SLM received care (11/11 at the CTE, 6/6 at the nursery, and 50/50 in the community). The province's total number of recoveries has reached 1,868.

- In North Kivu, 121 new admissions (17 confirmed and 104 suspected) were recorded, compared to 121 discharges and no recoveries. A total of 523 patients remained in isolation at the end of the day, representing an occupancy rate of 87.2% of Ebola Treatment Center (ETC)/Treatment Center (TCC) beds (326/374), including 19 critically ill patients at the Beni ETC; 148 patients (31.2%) are being treated in non-ETC/TCC facilities, and 91 patients, 12 caregivers, and 171 people with disabilities received hot meals.

- In Haut-Uélé, 71 new admissions were recorded compared to 4 discharges. In total, 71 patients remained in isolation at the end of the day, representing an overall bed occupancy rate of 53.6%. %; 2 critically ill patients; 50 patients, 65 accompanying persons and 70 PPL received hot meals.

- In Tshopo, 1 new admission (1 confirmed) was recorded. No discharges were reported. In total, 15 patients remained in isolation at the end of the day, representing an overall bed occupancy rate of 41.7% (15/36); since the beginning, 430 people have received the experimental treatment, including 37 confirmed cases, and 429 have benefited from post-exposure prophylaxis.

- In Bas-Uélé, 16 new suspected cases were admitted to transit centers (Bondo 7, Viadana 4, Ganga 3, Buta 1 and Dakwa 1), with no discharges; in total, 26 patients are reported in isolation, but the 2 new confirmed cases from the Ganga health zone are not yet isolated or taken care of and their safe transfer is urgently planned.

2.4.5. Challenges

- In Ituri, the low capacity of some CTE/CT (Mandima, Biakato and Lolwa), the care of 42 critical patients and the lack of food for 5 patients at the Mandima CT remain worrying.

- In North Kivu, the saturation of suspected beds (158/165; 95.8%), the care of 148 patients (31.2%) outside of CTE/CT and the low completeness of ambulance data (57%) remain worrying.

- In Haut-Uélé, the escape of 2 confirmed cases from the Pawa CTE, inconsistencies in care data and the absence of a hearse at the Boma Mangbetu CTE remain challenges to be solved.

- In Tshopo, the demotivation of 12 out of 29 ambulance workers (41.4%) undermines the transfer of patients.

- In Bas-Uélé, there is a lack of a standardized CTE (Emergency Medical Treatment Center), which is still under construction, and of an ambulance for transfers. Managing patients remains a major operational challenge.

2.5. Vaccination

2.5.1. Main actions

- In Bas-Uéléle, pre-registration of PPL and TPL (religious leaders, RECO, traditional healers, motorcyclists and hunters) continued, with the provincial cumulative total remaining at 1,500 people vaccinated out of a target of 13,202 (11.4%), mainly in Buta (620; 21.6%), in Ganga (273; 20.5%) and in Viadana (243; 9.8%); the provincial stock of Ervebo vaccine is exhausted (0 doses).

- In Haut-Uélé, 100 PPLs were vaccinated against Ebola on the first day of the campaign launched on October 8, 2026 by the Provincial Governor.

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- In Ituri, data collection for the study on the impact of the EVD epidemic on the use of sexual and reproductive health services continued in the health zones of Mongbwalu, Bunia, Rwampara, Tchomia and Nyarambe, as did qualitative and operational surveys and the Partners, Oracle and EBO-PEP clinical trials.

2.5.2. Challenges

- In Ituri, vaccination of PPL and the study of cross-immunity with the BDBV strain remain suspended pending the supply of Ervebo vaccine by the national PEV.
- In Bas-Uélé, the implementation of the UCCs is not effective and the micro-plan has not been validated, as well as the Vaccination funds are still not available.

2.6. Continuity of Care

2.6.1. Main actions

- In Tshopo, the evaluation of the RTNC health center revealed the absence of an effective triage area, service providers working without gloves, the absence of a functional incinerator, no service providers trained on Ebola and 5 agents vaccinated out of 28 (17.9%); the recovery plan for the Bondeko health center was developed, and the monitoring of PPL at the CTM and CTE as well as that of recovered patients with comorbidities at the Makiso-Kisangani General Referral Hospital continued.
- In Bas-Uélé, the evaluation of the Viadana General Referral Hospital shows a decrease in the use of services between May (7,009) and September 2026 (4,874), a bed occupancy rate of 20% in September for 70% expected, the absence of a triage and protection zone for PPL, and 16 PPL vaccinated out of 16.
- In North Kivu, ECZ members and ESS providers in the Beni health zone were briefed on the continuity of essential health services and the pillar participated in the launch, by IMC, of the project to maintain essential health services in the context of Ebola.
- In Bas-Uélé, no data has been transmitted by the continuity of care pillar in the last 24 hours.

2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

- In Ituri, 22,719 home visits reached 71,562 people; 614 educational talks brought together 3,307 people, 122 mass awareness campaigns reached 5,074 people, 18 community debates involved 639 people, and 9 advocacy sessions engaged 21 authorities and leaders; 21 public service announcements were broadcast on 36 radio stations, and 12 programs were organized. A total of 357 community alerts, including 127 deaths, were reported; 68 out of 83 feedback requests (81.9%) and 5 out of 7 incidents (71.4%) were resolved within 48 hours; 55 members of management committees, academic staff, and students in Bunia were briefed in preparation for the start of the academic year; and 229 contacts were pre-listed around 21 confirmed cases. The completeness of CREC reports is only 36% (10/28 ZS).
- In Bas-Uélé, 145 home visits reached 503 people in Likati, Monga, Ganga and Viadana, 2 educational talks reached 28 people and 2 mass awareness sessions reached 539 people; 77 people were sensitized around the positive cases in Dakwa and Ganga, 18 contacts were listed in Dakwa and advocacy was carried out with the chiefs of Kassa and Deni (Monga Health Zone).
- In Tshopo, 279 home visits reached 1,563 people, 3 educational talks reached 123 people and 8 mass awareness sessions reached 7,875 people, mainly in schools; 105 community alerts, including 1 death, were notified and all dealt with, the 3 feedback received were resolved (100%) and 125 contacts were followed up.
- In Haut-Uélé, 15 educational talks, 3 mass awareness sessions, and 122 home visits reached 2,712 people in Isiro, Wamba, and Watsa, and 42 community health workers and community health center members were trained; 54 community alerts, including 6 deaths, were reported in 5 health zones, and none of the 13 feedback requests received have yet been resolved. Vaccination of people living with HIV (PLHIV) against Ebola virus disease (EVD) was launched by the Provincial Governor in the presence of the Incident Manager.

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2.8. Mental Health and Psychosocial Support (MHPSS)

2.8.1. Main actions

- In North Kivu, 10 out of the last 20 confirmed cases (50.0%), 78 confirmed cases currently being monitored out of 129 (60.5%), 26 new suspected cases out of 81 (32.1%), and 89 suspected cases currently being monitored out of 120 (74.2%) benefited from SMSPS interventions. The teams supported 233 caregivers and 88 people living with HIV, monitored 19 out of 23 recoveries (82.6%) in the community and 14 out of 14 children in daycare, assisted 21 out of 25 separated and orphaned households (84.0%), reintegrated 15 out of 15 recoveries (100%), carried out 1 family reunification, and announced 45 results, including 10 positive ones; 113 children are being monitored.

- In Tshopo, 3 out of 4 most recent confirmed cases (75.0%), 5 confirmed cases under follow-up out of 5 (100%), 1 new suspect out of 1 and 7 suspects under follow-up out of 7 (100%) have benefited from SMSPS interventions; 4 households have been accompanied, 1 PPL supported, 1 recovered person followed in the community, 660 people have benefited from psychoeducation and 4 results, all positive, have been announced; 3 out of 4 resistances have been broken and 5 out of 7 health zones that notified are covered.

- In Haut-Uélé, 3 new confirmed cases, 23 confirmed cases under follow-up, 6 new suspected cases and 28 suspected cases under follow-up benefited from SMSPS interventions in the CTEs of Isiro, Pawa, Boma Mangbetu and Wamba; the teams supported 31 caregivers, 63 PPLs, 10 children separated in the community and 9 household members, supported 1 EDS and announced 19 results, including 3 positive ones.

2.8.2. Challenges

- In Ituri, only 14 out of 28 affected health zones are covered (50.0%) and the Mandima health zone has no SMSPS coverage, which requires strengthening the capacities of psychologists and APS/TPS to ensure interventions around cases.

- In Bas-Uélé, SMSPS coverage remains embryonic, with activities having only just begun in the Viadana ZS.

- In Haut-Uélé, the mobility of community health workers remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Wamba and Isiro, the community health worker lacks computer equipment, and data collection remains weak in Boma Mangbetu and Pawa

- In Tshopo, there is a need to train the APS of the ZS of Wanie-Rukula and Bafwasende and a shortage of communication units and motorcycles for the movement of the pillar.

2.9. Logistics

2.9.1. Main actions.

- Ensure the mobility of all field teams in all affected provinces.

2.10. Security

2.11. Main actions

- In North Kivu, public order was restored at the Bundji neighborhood office following an attack on the EDS team by the local population, with no damage reported; IPC teams were escorted to decontaminate four sites in the Kanzulinzuli health district and the La Modestine primary school, as well as a household in the Kasanga health district; the EDS team was escorted to the burial of a confirmed case in Lubero; and the FHI 360 partner was escorted to conduct a situation assessment in the Kambuli health district (Katwa). In Bas-Uélé, a security assessment of Dakwa and the road leading there from Buta was carried out, and a joint escort was formed to accompany the response team.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

- In Ituri, 33 service providers from the CTEs in Bambu, Nizi, and Medair (9 women and 24 men) were briefed on the PSEAH, and 55 people (24 women, 12 men, 16 girls, including 1 person living with HIV, and 3 boys) were sensitized on the PSEAH and reporting channels (151 and 495555) in the Mudzimaria, Kindia, and CNCA health districts (Bunia health zone); training for teachers, parent committees, community leaders, and women's associations was completed in Tchomia (20 participants, including 8 women and 12 men) and Komanda (57 participants, including 33 women and 24 men), and 1,420 students and staff from the Autsai and An-Nur schools in Aru (678 girls, 694 boys) were trained.

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(Boys, 19 women and 29 men) were sensitized. The display of messages and the distribution of leaflets continued in Bunia, Aru, Mongbwalu, Mambasa and Fataki.

- In North Kivu, 76 people (56 men, including 3 people living with HIV, and 20 women) were briefed on the PSEAH in Beni during 4 sessions (19 SMSPS providers, 20 Red Cross volunteers and 37 local providers and leaders), and 140 people were sensitized on PSEAH and reporting via 151/495555 in Beni (64, AS Paida and Tuungane), Katwa (54, AS Vungi) and Butembo (22, AS Vulind

2.12.2. Challenges

- At the national level, 4 out of 7 DPS (57.1%) reported their PSEA activities (Ituri, North Kivu, Haut-Uélé and Bas-Uélé); in Ituri, only 13 out of 36 ZS (36.1%) know the reporting mechanism, only one partner (WHO) reports its activities and awareness materials and computer kits are lacking.

MESSAGES CLES

ÿ In the provinces of Ituri, North Kivu and Haut-Uélé, it is observed active community transmission while sporadic notification is observed in Tshopo province.

2. In the last 24 hours.

ÿ Over the past 24 hours, all suspected cases (100.0%) have been investigated; performance is consistent with data reported by the provinces. However, contact tracing remains inconsistent (83.1% vs. 83.5%) and warrants close attention to prevent uncontrolled chains of transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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